



PMSMA & EPMSMA

Topic: 6

Pradhan Mantri Surakshit Matritva Abhiyan (PMSMA)/Ext ended PMSMA

PMSMA is Launched in 2016 by the **Ministry of Health & Family Welfare** (MoHFW), Government of India. And E-PMSMA 1st April 2022

PMSMA **provides fixed-day assured**, comprehensive and quality antenatal care universally to all pregnant women (in 2nd and 3rd trimester) on the 9th of every month and E PMSMA On any other Fixed days of the months Other than 9th.

On this day special ANC services are provided by **OBGY specialists/ Radiologist/ Physicians** at government health facilities.

Using the principles of a single window system

One of the critical components of the Abhiyan is identification and follow-up of high-risk pregnancies and red stickers are added on to the Mother and Child Protection cards of women with high-risk pregnancies.

WHEN



9th of every
month and Any
Other Fixed Day
(E-PMSMA)

BY
WHOM



BEmONC
trained or
specialist

FOR
WHOM



2nd & 3rd Trimester
PW

Features



MOBILIZATION



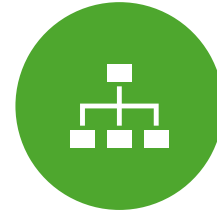
SCREENING



HRP
IDENTIFICATION



PMSMA
PORTAL



TRACKING OF
HRP



MANAGEMENT
OF HRP

Financial Support



ASHA :

- Mobilization Support to ASHA on PMSMA Rs. 100/-
- HRP Mobilization support to ASHA RS.100/- for maximum 3 visit (Rs. 300/-)
- Positive Outcome of Mother and child on 42 days Rs. 500/- Per High Risk

Beneficiary:

- HRP transport support to PW RS.100/- for maximum 3 visit (Rs. 300/-)

Non- Negotiable Component

Availability of Specialist (Obs & Gyn)/ BEmONC

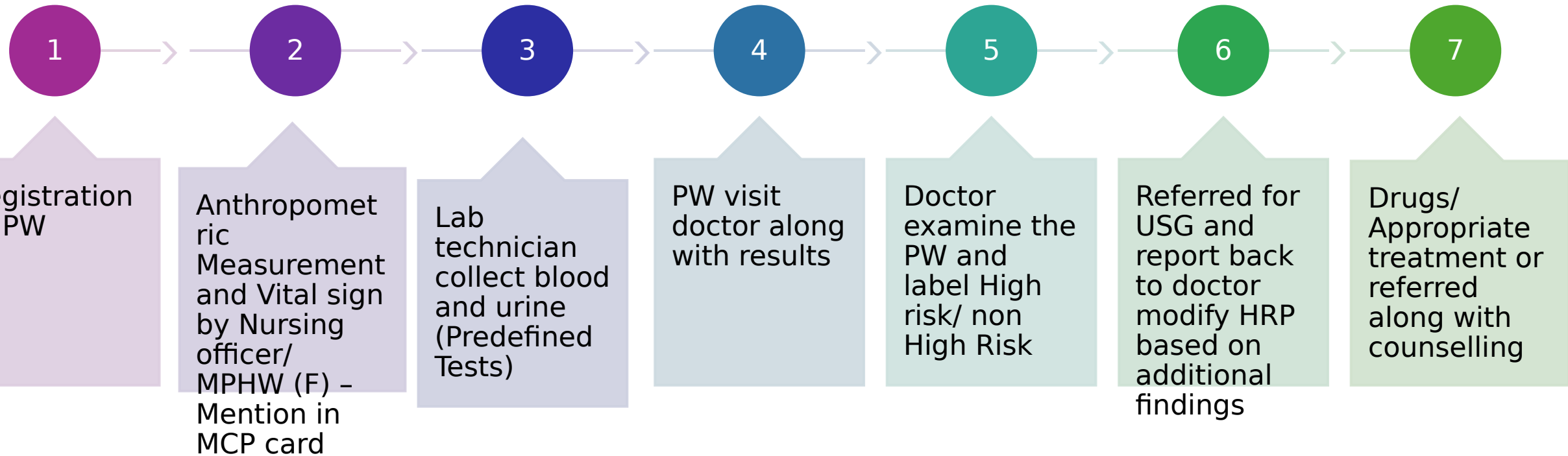
History Taking (All the History as per MCP card)

Investigations (Hemoglobin, Urine albumin & Sugar, OGTT, Rapid malaria Test, CBC, Blood grouping, TSH & SCD)
USG Coverage (Mandatory 1 USG for all PW)

Diet (Hot cooked Meal)

Flow of Beneficiary

Flow of Beneficiary



Roles and responsibility of Health care providers

Obstetricians/ BEmONC trained MO:

- Conduct thorough ANC check-ups.
- Identify and manage high-risk pregnancies.
- Ensure accurate documentation and reporting.

Nursing Officer and MPHW (F):

- Assist in ANC procedures.
- Facilitate patient flow and comfort.
- Provide health education to pregnant women.

ASHA Workers:

- Mobilize pregnant women to attend PMSMA clinics.
- Maintain line lists of pregnant women.
- Follow up with high-risk cases.

List of PMSMA HRP Condition (25)

S.L. No	HRP Condition	S.L. No	HRP Condition
1	HIV	14	Abnormal FHR
2	Syphilis	15	Teenage Pregnancy
3	Severe Anemia	16	High Fever
4	PIH	17	RTI/STI
5	GDM	18	H/o Still Birth
6	Hypothyroidism	19	Congenital Malformation
7	Tuberculosis	20	Negative Blood Group
8	Malaria	21	Early primi
9	Previous LSCS	22	Elderly Primi
10	CPD	23	Grand multipara
11	BOH	24	Short Stature
12	Twins/Multiple Pregnancy	25	Others Specify
13	Hepatitis B		

Integration of MCP card and PMSMA HRP condition

S.L. No	PMSMA HRP Condition	MCP card HRP Alignment
1	HIV	HIV
2	Syphilis	
3	Severe Anemia	Heamoglobin lessthan 7gm%
4	PIH	BP >140/90mmhg, Urine albumin
5	GDM	Urine sugar, OGTT
6	Hypothyroidism	Thyroid prblem
7	Tuberculosis	H/O TB
8	Malaria	Malaria
9	Previous LSCS	Previous LSCS
10	CPD	
11	BOH	Prev. LBW, Preterm, IUGR, newborn deaths, Previous H/O APH , H/O PPH, morethan 3 abortion, H/O PIH or eclampsia, primary / Secondary infertility, IVF
12	Twins/Multiple Pregnancy	
13	Hepatitis B	Hepatitis B

Integration of MCP card and PMSMA HRP condition

S.L. No	PMSMA HRP Condition	MCP card HRP Alignment
14	Abnormal FHR	
15	Teenage Pregnancy	If pregnancy less than 18 years
16	High Fever	Fever
17	RTI/STI	RTI/STI
18	H/o Still Birth	Previous Still birth
19	Congenital Malformation	
20	Negative Blood Group	Rh -ve
21	Early primi	If pregnancy less than 18 years
22	Elderly Primi	If 1 st pregnancy morethan 35 years
23	Grand multipara	More than 4 pregnancy
24	Short Stature	Height lessthan 140cms
25	Others Specify	Geographically Hard to reach area, Prev. sx involving Uterus, H/O Diabetes, Heart disease, Kidney disease, H/O Leprosy. Any Pregnancy morethan 35 years, Thalassemia, SCD, weight lessthan 40kgs,ASB, inappropriate wt gain , MUAC Lesstha 23cm